

FDA DICE Call — Summary

Date: September 1, 2026 **Participants:** Isak Gutierrez, Joelle Ater, FDA DICE agent

Key Findings

1. If it lifts, it's a lift

The FDA agent was clear: **if the device has lift capability, the agency would still consider it to be a lift**, regardless of whether the primary function is walking/therapy. Marketing it as “just a walker” while it has a powered lift column does not avoid the lift classification. Intended use matters, but physical capability matters too.

2. Dual product codes are allowed

A device with more than one function can have **more than one product code** — a primary and secondary. The FDA guidance on product codes explains this “subsequent product codes” concept. The company determines the primary code.

3. No pre-market authorization needed for any of the lift codes

- **FSA** (Lift, patient, non-AC-powered): Class I, 510(k) exempt
- **FNG** (Lift, patient, AC-powered): Class II, but **510(k) exempt**
- Neither requires a pre-market submission before selling

4. The 513(g) process exists but is optional

For \$7,820 (or \$4,298 small business), the FDA will give a **suggested classification** — not binding, but an official artifact. It's a convenience, not a requirement.

5. Risk of wrong classification

If you classify under a code that doesn't require pre-market authorization but the FDA later determines it should have been under one that does, they can **make you stop selling** until the correct submission is filed and authorized.

What This Changes

The ITJ-only strategy is weaker than we thought. The FDA agent essentially said that a device with a powered lift column cannot avoid the lift classification through marketing language alone. This aligns with what the decision tree flagged as the root question.

However, the practical impact may be smaller than feared: - FSA (the most likely lift code) is still Class I, still 510(k) exempt - The difference is **GMP requirements** — FSA requires a full quality system (ISO 13485-level), while ITJ is GMP-exempt - No pre-market submission needed either way

The Biodex BXB (powered exerciser) path is still worth exploring — the FDA agent didn't mention it, and it wasn't asked about directly. A 513(g) submission would settle this definitively.

Recommended Next Steps

1. **Read the two guidance documents** the agent referenced (513(g) procedures + product code classification guidance)
2. **Decide whether to file a 513(g)** — \$4,298 small business fee, gets an official FDA position. This was test #4 in the decision tree (regulatory consultant memo), and the FDA offers it directly.
3. **Pull FDA registrations for Biodex, Rifton, Exokinetics** — still test #1 in the decision tree, still free, still unexecuted
4. **Share this summary with Thomas** — his instinct to go with the heavier classification may be validated by this call

Documents to Review

- 513(g) Guidance — search “513(g)”
- Product Code Classification Guidance — search “product code”
- Device Advice — Regulatory Controls — general controls + special controls overview